



Progressive Risk Assessment SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD) inpatients only	Medical, Nursing, Allied Health

Purpose

This standard operational procedure (SOP) provides guidelines and defines the scope of practice for multidisciplinary team (MDT) members using the Progressive Risk Assessment (PRA) to promote safety for patients and clinicians.

PRA would be used only if patient is on a mental health ward or service.

Links within document

- [Appendix I: Progressive Risk Assessment Guide](#)

Definitions

At risk	A patient whose physical condition or mental health state increases the probability of clinical deterioration and the likelihood of an adverse event occurring.
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When to complete a PRA

Complete the PRA:

- On ward transfer within RPBG
- When patient and/or personal support person (PSP) requests
- When a new risk factor is identified (i.e. harm to self, infection)
- Post incident (i.e. fall, seclusion, self harm)
- On condition change (i.e. decreased Glasgow Coma Scale (GCS) due to pharmacological management, adverse effects due to clozapine)
- On medical team review.

If a new Risk Assessment Management Plan (RAMP) is completed, for the below reasons, a PRA is not required at that time.

- Following change in Mental Health Act (MHA) 2014 legal status
- On leave: refer to Leave Management for Mental Health (MH) Inpatients Policy
- When transport and/or escort is required
- When change to psychosocial factors (i.e. loss of job)
- When change in mental state
- On team review (i.e. MDT meetings, MH Tribunal meetings)

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When to complete a PRA cont'd

Note: The Transitional Care Unit (TCU) is only required to use PRA at certain instances of leave (i.e. patient going on day leave, overnight leave, weekend leave or longer, as clinically indicated).

Completion of the PRA

- Any member/s of the MDT can complete the PRA; however, it is recommended that this is done as an MDT, especially post incidents or near misses
- A PRA should be guided by findings from:
 - The latest clinical handover
 - Relevant documentation (i.e. patient healthcare record entries)
 - The RAMP, in conjunction with the Treatment, Support and Discharge Plan (TSDP) and the Multidisciplinary Comprehensive Care Plan (MCCP).
 - Clinical presentation of the patient and/or PSP
- One of the following options must be selected against each risk based on the clinician's assessment (refer to [Appendix I](#)):
 - No - nil evidence
 - Yes - requires monitoring and intervention to minimise risk
 - Yes - definite risk requiring immediate intervention
 - Unable to confirm (specify)
- For each new risk identified the clinician must document on the PRA relevant findings and observations to support the determination of that risk
- Document on PRA interventions or actions to minimise the risk(s) identified, which prompts change in patients' management in the patient's healthcare record, TSDP and What Matters Most Record (WMMR)
- When completing the PRA, clinicians are to indicate:
 - Current MHA 2014 status
 - The leave status. Refer to **Leave Management for MH Inpatients Policy**
 - The level of observations (refer to the **Special and Supportive Observations Policy**)
 - Any changes from the previous PRA
- Complete the PRA in the patient's healthcare record and document information that supports the management plan
- The most recent PRA must be actively communicated and given prominence in clinical handovers, safety huddles, team discussions and/or to the patient and PSP
- The MDT are responsible for working collaboratively to continually screen, assess and monitor the patient for risks and update appropriate interventions in the TSDP, WMMR and PCP until the patient is transferred or discharged from the service.
 - Consider cultural needs and/or risk: e.g. patients who identify as a member of the LGBTIQ+ community, Aboriginal people.

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Compliance monitoring

Compliance will be monitored during clinical incident investigations.

Audits as per the RPBG Performance Monitoring Schedule and reported at the MHD Clinical Quality and Safety meetings.

Authors / acknowledgements

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Related policies, practice standards, clinical guidelines

[RPBG Policy HUB](#)

- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Minimum Requirement for Medical Review of Consumers Admitted to Inpatient Wards/Community Mental Health Services Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Physical Restraint and Seclusion in Mental Health Policy](#)
- [LGBTIQ+ Inclusivity Policy](#)

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Related policies, practice standards, clinical guidelines cont'd

EMHS Policies

- [Aboriginal Cultural Group Policy](#)
- [Risk Management Policy](#)
- [Rights of Patients in Mental Health Policy](#)

WA Department of Health Policies Hub

- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- [State-wide Standardised Clinical Documentation \(SSCD\)](#)
- Psychiatric Services Online Information System (PSOLIS) MHA 2014 Business Operating Rules

Other

- *Mental Health Act 2014*
- Chief Psychiatrist's Standards for Clinical Care
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021);

Standard 1: Clinical Governance;

Standard 2: Partnering with Consumers;

Standard 5: Comprehensive Care,

Standard 6: Communicating for Safety;

Standard 8: Recognising and Responding to Acute Deterioration

NMHS Standards (2010):

Standard 1: Rights and Responsibilities;

Standard 2: Safety;

Standard 4: Diversity Responsiveness;

Standard 6: Consumers; Standard 7: Carers;

Standard 8: Governance, Leadership and Management;

Standard 10: Delivery of Care

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Appendix I: Progressive risk assessment guide

Consider other risks and vulnerabilities e.g. domestic violence, elder abuse, physical or intellectual or cognitive or developmental disabilities in the PRA

Risk	Yes - requires monitoring and intervention to minimise risk	Yes – definite risk requires immediate intervention
Leaving without notice	- Past history, current thoughts of absconding - Able to guarantee that they will not leave ward	- Recent history - Unable to guarantee that they will not leave ward - Refuses to seek staff assistance - Altered mental state - Lack of insight - Current thoughts regarding leaving without notice and with plans
Harm to self	- Current thoughts of self-harm, able to guarantee safety - Recent minor self-harm, regretful and able to guarantee safety - Past history	- Significant self-harm - Current self harm thoughts and/or plans, unable to guarantee safety and/or has access to available means and/or unable to seek staff assistance - History of impulsivity - Labile mental state - Unable to engage with team - Lack of insight
Harm to others	- Past history of harm to others - Current thoughts of harm to others, able to guarantee they will not act on thoughts	- Recent history of harm to others - Current thoughts or plans of harm to others or unable to guarantee that they will not act on thoughts - Current delusions or intrusive thoughts related to personal safety or danger towards others - Labile mental state - Unable to engage with team - Lack of insight
Harm from others	- Past history of harm from others - Physical or developmental disability	- Past or recent history of harm from others - Altered mental state causing disinhibition or acute behavioural disturbance <18 years of age - Domestic violence - Sexual vulnerability
Physical health risks	Pressure injuries, falls, getting lost, delirium and/or cognitive impairment, alcohol and/or drug and/or nicotine withdrawal, Dysphagia and/or aspiration, malnutrition, venous thromboembolism, infection	

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